Abiraterone + prednisolone

To control prostate cancer and delay progression; your team explains the expected benefit for your situation.

Newly diagnosed high-risk metastatic hormone-sensitive prostate cancer · NCCP 00577 v3 · Document revised 14 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Newly diagnosed high-risk metastatic hormone-sensitive prostate cancer

My overall treatment

Daily tablets plus ongoing ADT

28 day reference

Reference window · ongoing treatment

Abiraterone + prednisolone every day — Day 1–28

Treatment

Abiraterone 1000 mg once daily plus prednisolone 5 mg once daily, continuously while helping and tolerated. Continue your ADT.

Review

Continue ADT. The team reviews symptoms, blood tests, PSA and scans and confirms whether treatment continues.

Your 28-day review window

Day 1–28

Every day, including weekends

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline preparation** NCCP	Full blood count, kidney and liver tests, glucose and blood pressure; electrocardiogram (ECG) if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.
During	**Safety checks** NCCP	Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.
Response	**Cancer-specific checks** Individual specialist plan	PSA and testosterone assess cancer activity and hormone suppression; scans follow your individual plan.

Between treatments

- Symptoms can occur on any day; keep your urgent contact plan.
- Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.

Bring your medicines list, calendar and verified contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy, balance and thinking

Abiraterone + prednisolone

Report troublesome fatigue, falls or confusion. Seizure, collapse or sudden neurological symptoms need emergency help.

3 Heart and blood pressure

Abiraterone + prednisolone

Keep blood-pressure checks. Chest pain, fainting or severe breathlessness needs urgent help; report ankle swelling or palpitations.

5 Appetite and bowels

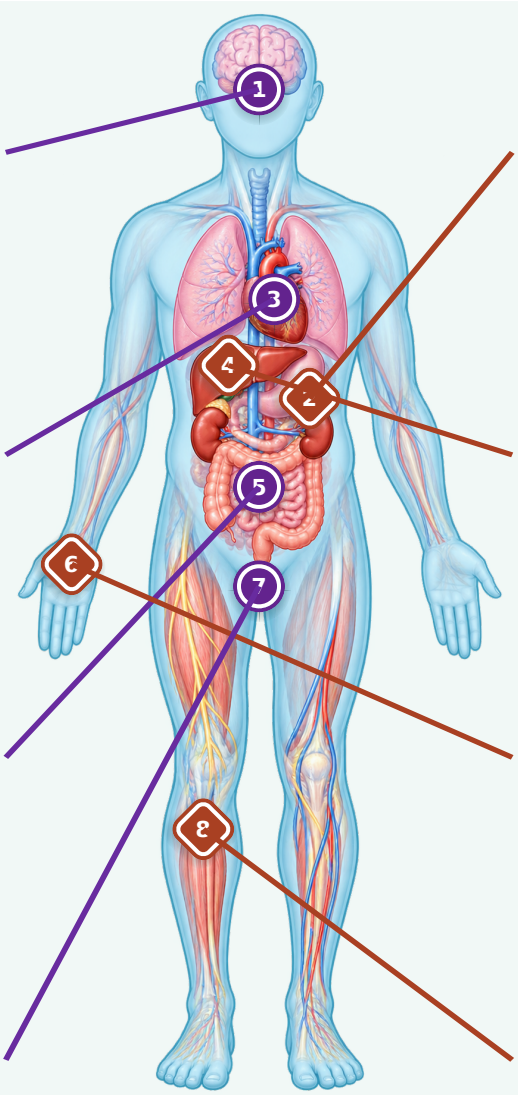
Abiraterone + prednisolone

Report persistent vomiting or diarrhoea, inability to drink or take medicines, and unintended weight loss.

7 Sexual health and fertility

ADT and Abiraterone + prednisolone

Discuss erections, libido, fertility and contraception. Avoid exposing a pregnant partner to these medicines; follow the product-specific precautions.



2 Steroid and potassium balance

Abiraterone / prednisolone

Keep prednisolone as prescribed. Do not stop abruptly. Low potassium may cause weakness or palpitations; vomiting or serious illness needs a steroid plan.

4 Liver and blood tests

Abiraterone + prednisolone

Keep liver blood tests. Report yellow skin or eyes, dark urine, reduced appetite or a major change in wellbeing.

6 Skin changes

Abiraterone + prednisolone

Report a new rash. Severe rash with blisters, peeling, fever or mouth/eye sores needs urgent assessment.

8 Bones and muscle strength

ADT and Abiraterone + prednisolone

Discuss bone-density assessment, falls prevention, suitable resistance and aerobic activity, calcium intake and vitamin D needs.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for sudden illness, persistent vomiting, inability to drink, jaundice, new breathlessness or severe rash. Call 112/999 for severe chest pain, severe breathing difficulty, collapse, seizure, or new leg weakness with loss of bladder or bowel control.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Abiraterone + prednisolone

Daily tablets plus ongoing ADT

Phase: _____ Continuous treatment: team reviews dose

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My review record

Recording spaces only; not a fixed course.

Review	Date / initials

My treatment information

Latest review date: _____

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team records each review and the current daily dose. Recording boxes do not imply a fixed course or a rest period. Record any held dose or changed plan. An entry does not authorise a restart; keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.